

Health Content Check

The one-page version. For anyone writing health information for patients or the public. No AI needed. Print it, pin it.

First, the one that decides everything

Is this page information, or is it marketing? If its job is to persuade someone to book, buy or choose a provider, it is marketing. It cannot be assessed as patient information, and dressing it up as information is the most damaging pattern on healthcare websites. Plain language and a tidy reference list make a sales page more persuasive, not more trustworthy.

Signals: superlatives about the provider, booking as the main call to action, outcomes with no source or no denominator, testimonials doing the work evidence should do. Split the selling onto its own page.

Check these on the page itself

- [] **One clear job.** You can say in a sentence who it is for and what decision it helps them make.
- [] **It answers the awkward questions.** Does it hurt. How long off work. What if I do nothing. What does it cost.
- [] **Every number has a source, a denominator and a date.** If you cannot source it, remove it.
- [] **Frequencies, not percentages.** "About 1 in 200 people" beats "0.5%". Same denominator throughout.
- [] **Risks as well as benefits,** including rare things that are serious.
- [] **Doing nothing is covered,** honestly, alongside the conservative options.
- [] **Uncertainty is stated.** "We do not know" is a legitimate sentence and a trust signal.
- [] **Short sentences and short paragraphs.** About 20 words. About 3 sentences.
- [] **Every technical term is explained or replaced,** on first use.
- [] **Named author with their role,** the organisation, the date written, the review date and any commercial interest, all visible to the reader.
- [] **A route to a real person,** what to do urgently if things go wrong, plus links out to the national source even when it is not yours.
- [] **Alt text on images,** captions on video, links that say where they go.

Only you can confirm these. Do not tick them lightly.

- [] A qualified clinician has checked the clinical content. If that is you, someone else still reads it.
- [] A lay reader who does not work in healthcare has read it and understood it.
- [] Patients or members of the public were involved, or at least asked.
- [] There is a way for readers to say it is wrong or unclear, and someone reads it.
- [] The review date is diarised, owned by a named person and honoured.
- [] Someone knows whether it is read, and whether it helped.

Asking two patients what they did not understand finds more than any automated score.

When it is all wrong, fix in this order

1. Anything that could lead a reader to a harmful decision: a wrong number, an overclaim, a missing serious risk, no route to urgent help. 2. Marketing dressed as information. 3. Missing author, date or review date. 4. Readability. 5. Everything else.

What this is not

Not certification. No badge, tick or logo, and not run or endorsed by NHS England or by the Patient Information Forum. Quality marks are awarded by schemes that assess how you produce content over time, not by anything that inspects a single page. What you can honestly say: *written to the NHS standard for creating health content.*

Built on the NHS digital service manual standard and the 10 published PIF TICK criteria. Free to use and adapt: github.com/alistairphillips1/health-content-check